



Off-label psychotropic prescribing guide for children

Policy code	SMHS: 168
Original endorsement date	December 2015
Date of current endorsement	August 2025
Functional sub group	Mental health
Summary	This policy is to be used as a quick-reference guide to determine the licensed age, indication and dosage for psychotropic medication use in paediatric patients. The guideline also contains a very brief description of the level of evidence for use of the psychotropic medication in major international clinical guidelines.
Policy owner	Executive Director Area Mental Health
Contact	Peta Williams
Review date	March 2026
National Safety and Quality Health Service (NSQHS) Standards	Standard 1: Clinical Governance Standard 4: Medication Safety
National Standards for Mental Health Services 2010 (NSMHS)	Standard 2: Safety Standard 10: Delivery of care
Status	Active

1. Background

The *Mental Health Act 2014* (MHA 2014) provides that a record must be made of off-label prescribing to a child (a person who is under 18 years of age) who is an involuntary patient. South Metropolitan Health Service (SMHS) clinicians are required to obtain consent for the use of off-label medication from the patient's parent or guardian when prescribing off-label treatments, irrespective of the patient's status under the MHA 2014.

Off-label treatment means the use of registered therapeutic goods other than in accordance with the approved product information for the registered therapeutic good. If the medication is being used outside of the age group, dose, indication or route as mandated by Therapeutic Goods Administration registration and/or official manufacturer's product information then it constitutes off-label prescribing.

This policy is to be used as a quick-reference guide to determine the licensed age, indication and dosage for psychotropic medication use in paediatric patients. The policy also contains a very brief description of the level of evidence for use of the psychotropic medication in major international clinical guidelines. It is not intended to be used as a primary reference for medication dosages.

The policy should be used in conjunction with [Appendix A – Flowchart for assessing appropriateness of off-label medicines use](#).

1.1. Key definitions

Abbreviation	Full Term
ADHD	Attention Deficit Hyperactivity Disorder
ASD	Autism Spectrum Disorders
AUT	Autism
BD	Behavioural Disturbance in Intellectual Disability
BPAD1	Bipolar Affective Disorder 1
Dx	Diagnosis
eTG	Electronic therapeutic guidelines
GAD	Generalised anxiety disorder
MD	Major Depression
NICE	National Institute for Health Care and Excellence
NP	Neuropathic pain
OCD	Obsessive Compulsive Disorder
PD	Panic Disorder
PMDD	Premenstrual Dysphoric Disorder
PS	Phobia syndromes
PTSD	Post-Traumatic Stress Disorder
SAD	Social anxiety disorder
SCZ	Schizophrenia
SZ	Seizure disorder

Most non-psychiatric indications have been left off tables for ease of reading.

2. Responsibilities of SMHS Clinicians

To assist clinicians to make informed decisions about prescribing off label, and to create a platform to obtain informed consent the following resources have been developed by Child and Adolescent Mental Health Service and will be placed in the [CAMHS Medication Safety Toolbox](#).

FSH consent form for off-label medication use is available from PMG – [Appendix B](#).

As a SMHS clinician you have a professional, ethical and legal obligation to:

- evaluate the evidence for yourself;
- inform the patient and parent/guardian that the medication is off-label;
- discuss alternatives;
- provide the patient and parent/guardian with consumer product information, to provide an opportunity to study this information and to answer any questions;
- obtain written parental/carer consent; and
- comply with the recording and reporting requirements set out in section 304 of the MHA 2014.

3. Antidepressants

	Australian Licensed Use for under 18's	Dx	Dose (daily)	NICE	Cochrane	eTG
Citalopram	No			second line MD	nil	2nd line MD
Escitalopram	No			nil	nil	nil
Duloxetine	No			nil	nil	nil
Fluoxetine	No			MD, OCD if also depressed	MD	8+
Fluvoxamine	8+	OCD	25- 300mg		nil	nil
Paroxetine	No			on nice 'do not do' list	nil	not recommended
Sertraline	OCD 6+	OCD	25- 200mg	2nd line MD, OCD	weak	2nd line

Venlafaxine	No	on nice 'do not do' list	nil	Should not be used in children -increased self-harm and SI, hostility
Clomipramine	No	nil	nil	not recommended
Desvenlafaxine	No	nil	nil	not recommended
Mirtazepine	No	nil	nil	not recommended

4. Antipsychotics

	Australian Licensed Use for under 18's	Dx	Dose (daily)	NICE	Cochrane	eTG
Aripiprazole	No			B1AD 13+, SCZ 15+, AUT after psychosocial	Autism	nil
Olanzapine	No				psychosis in adolescents	Eating disorders
Quetiapine	B1AD 10+, SCZ 13+	B1AD, SCZ	50-750mg		nil	nil
Risperidone	SCZ 15+, BD 5+, AUT 0+	SCZ, BD, AUT	0.125-8mg daily, 0.01-0.06mg/kg/day	AUT after psychosocial Tx. for ASD eg BD-shortterm after psychosocial	small evidence for ASD eg aggression	ASD, Tourettes, disruptive behaviour
Clozapine	16+	SCZ	25-300mg		very weak	nil
Chlorpromazine	children age unclear	BD, AUT		nil	nil	nil
Flupenthixol	No				nil	nil
Zuclopenthixol	No				nil	nil

5. Mood stabilisers

	Australian Licensed use for under 18's	Diagnosis	Dose (daily)	NICE	Cochrane	eTG
Lamotrigine	2+ SZ,	SZ	50-400mg	nil	nil	not psych
Lithium	12+	B1AD, SCZ and related	500-2000mg	nil	nil	nil
Carbamazepine	0+	SZ	200-1600mg	nil	nil	not psych
Sodium Valproate	SZ	SZ	600-2000mg	nil	nil	not psych

6. Stimulants

	Australian Licensed Use for under 18's	Diagnosis	Dose (daily)	NICE	Cochrane	eTG
Methylphenidate	6+	ADHD, narcolepsy	5-60mg	ADHD	ADHD	ADHD
Dexamphetamine	3+	ADHD, narcolepsy	2.5-40mg (ADHD) 2.5-60 (Narcolepsy)	ADHD	nil	ADHD

7. Other

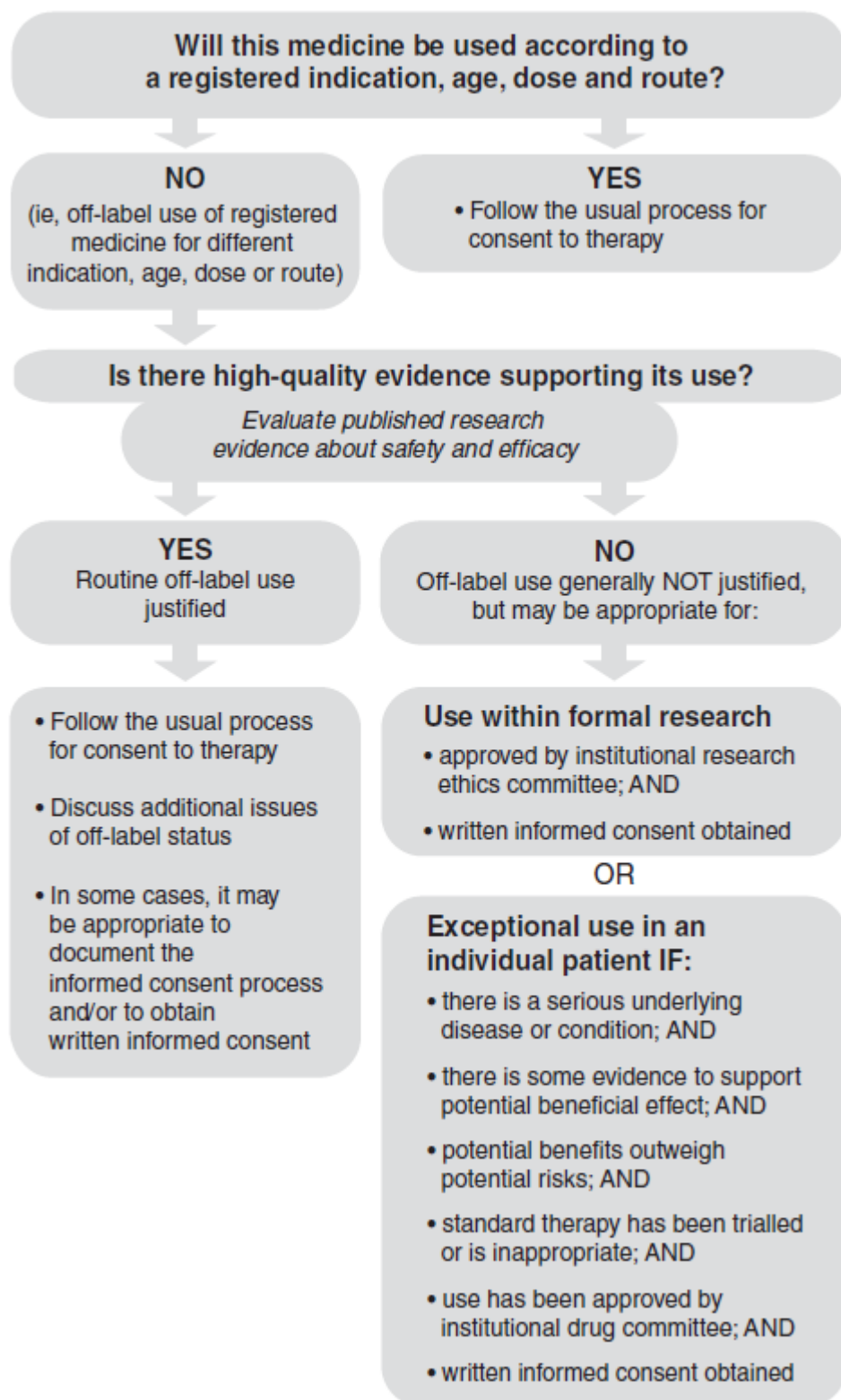
	Australian Licensed Use for under 18's	Diagnosis	Dose (daily)	NICE	Cochrane	eTG
Atomoxetine	6+	ADHD	up to 1.4mg/kg or 100mg	ADHD	tics, ADHD	ADHD
Melatonin	No			nil	nil	nil
Lorazepam	16+	GAD	1-10mg		nil	nil
Clonazepam	2+	SZ	1.5-8mg		nil	not psych
Diazepam	6months+	anxiety, muscle spasm, agitation	1-40mg		nil	not psych
Zolpidem	No				nil	nil

8. Additional references

- Refer to Office of the Chief Psychiatrist [Monitoring and Reporting](#)
 - Refer to Office of the Chief Psychiatrist [Chief Psychiatrist's Standards for Clinical Care](#)
 - [MIMS Online: Approved Product Information Database](#)
 - [CAMHS Psychotropic Medication – Monitoring Adverse Physical Health Effects Policy](#)
 - [Psychotropic Medication: Physical Health Monitoring Handbook CAMHS](#)
 - [CAMHS Medication Safety Toolbox](#)
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- Please note this policy was endorsed for a 4-month extension. New review date will be March 2026. Endorsed at AEG meeting 12 August 2025.

9. Appendix A: Flowchart for assessing appropriateness of off-label medicines use

2 Assessing appropriateness of off-label medicines use



10. Appendix B: Fiona Stanley Hospital Consent form for off-label medication use

Please use I.D. label or block print

FIONA STANLEY HOSPITAL FIONA STANLEY YOUTH HEALTH CONSENT FOR OFF LABEL MEDICATION USE WARD _____ DOCTOR _____	SURNAME _____	UMRN _____	
	GIVEN NAMES _____	DOB _____	GENDER _____
	ADDRESS _____		POSTCODE _____
	TELEPHONE _____		

This form is to be completed giving due consideration to the 'Consent to Treatment Policy for the Western Australian Health System'

☐ I have talked about the nature of _____'s condition and the use of medicine/medications as one part of the overall treatment plan
☐ I understand that the use of _____ will be 'off label'. "Off label" means it is not registered for use with the Therapeutic Goods Administration in Australia for this age group.
☐ I have discussed the nature of the evidence available for its use with the prescribing doctor
☐ I have been given consumer product information
☐ I have been informed of the benefits and risks related to the use of this medicine
☐ I will ensure the medicine is not taken outside of the prescribed guidelines during the treatment period
☐ I have been given the opportunity to have any questions and concerns clarified
☐ I understand that I have the right to withdraw consent at any time, including after I have signed this form and that I must inform the FSH mental health clinician if I change my mind

Details of off label medication

Generic name _____ Dosage: _____

Reason for use: _____

Disclosure of material risks _____

Serious risks: _____

Declarations _____

Parent/Guardian/Mature Minor's (full name) _____

Relationship to child/minor: _____

Date ____/____/____ Time ____:____ am / pm Signature _____

Psychiatrist (full name)

Date ____/____/____ Time ____:____ am / pm Signature _____

Interpreter's declaration

Interpreter services required ☐ Yes ☐ No

Specific language requirements (if any) _____

Interpreter Service used ☐ On site ☐ Telephone (record interpreter's full name and NAATI number)

I declare that I have interpreted the dialogue between the parent/guardian and health professional to the best of my ability, and have advised the health professional of any concerns about my performance.

Interpreter's Full name: _____ NAATI number _____

Date ____/____/____ Time ____:____ am / pm Signature _____

FS600300
 DO NOT WRITE IN MARGIN
 HC-F-SMR869
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MR 869 CONSENT FOR OFF LABEL MEDICATION USE

Acknowledgment to CAMHS

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